

إسم المريض رابعياً : منى رضوان سعيد عبد اللطيف

المحافظة : الفيوم الحي / المدينة : يوسف الصديق العنوان : المشترك البحري

رقم البطاقة : 28107032300825 رقم التليفون : 01105895541 تاريخ تسجيل الإستمارة : 01-10-2026

العمر(تاريخ الميلاد) : 03-07-1981 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضى الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع: 

التوقيع: (الدي حيدر)

التوقيع: (مستشار)

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: منى رضوان سعيد عبد اللطيف National ID: 28107032300825	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



فتح طارق
 د. طارق عبد طيب
 ١٤١٧



Patient Name : الأستاذ / منى رضوان سعيد عبد اللطيف	Request No. : 13003414
Patient No. : 28107032300825	Request Date : 15/11/2025
Path No. : SR-1646/25	Verification Date : 01/12/2025
Sex : Female	Financial Category : جوسف روسي - نفقة الدولة
Age : 44 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

IPT:

* ER: Positive (score 8/8).

* PR: Positive (score 8/8).

* HER2: Negative (score 0)

* Ki67: 25 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 07/12/2025 10:17:02

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: مني رضوان سعيد عبداللطيف			
MR. : 145			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 45-year-old premenopausal individual with right-sided breast carcinoma, invasive ductal carcinoma, grade 2, estrogen receptor 8/8 positive, progesterone receptor 8/8 positive, HER2 negative, and Ki-67 30 %. Staging work-up included a CT scan of the chest, abdomen, and pelvis, which was negative for distant disease, followed by a bone scan that was free of lesions. Magnetic resonance imaging of the bones identified possible metastatic involvement of the left femur and tibia. The patient was referred to a multidisciplinary tumor board and initiated on an aromatase inhibitor combined with Zoladex and Zometa for three months, with plans for reassessment thereafter. The patient was continued on Femara (letrozole) plus Zoladex with supportive care for a three-month interval, and a follow-up clinic visit was scheduled for 1 October 2026.			
تحريرا في ,			
التاريخ :		20-09-2026	
الوقت :		mohamed ismael	
		الطبيب المعالج :	
يعتمد ,			

Patient Name : الأستاذ / منى رضوان سعيد عبد اللطيف	Request No. : 13003414
Patient No. : 28107032300825	Request Date : 15/11/2025
Path No. : SR-1646/25	Verification Date : 01/12/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data

Right breast mass.

Nature of specimen

Block revision

Gross examination

Five slides labelled by (15259/25) were received . . .

Microscopic examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Positive , cribriform subtype , intermediate grade
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification :Absent
- * Perineural invasion: Absent
- * IPT:
- * ER: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * HER2: No membrane reaction in tumor cells (score 0)
- * Ki 67: 25 %

Diagnosis

RIGHT BREAST MASS, BLOCK REVISION

-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer) , with duct carcinoma insitu ,
cribriform , moderate grade

Verified by :

Dr.Heba Hasan

Printing Date : 07/12/2025 10:17:02

Page 1 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / منى رضوان سعيد عبد اللطيف	Request No.	: 13003414
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Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Verified by :
Dr.Heba Hasan

Printing Date : 07/12/2025 10:17:02

Page 3 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Tc-99m MDP whole-body bone scan

PATIENT NAME: MONA RADWAN SAED

DATE OF SCAN: 09/11/2025

History: 45-year-old female presenting with history of breast cancer, referred for skeletal survey.

Technique: Anterior and posterior whole-body scans were acquired 2 – 3 hours post injection of 20 mCi of the tracer.

Findings:

- The shafts of both the left femur and tibia show scattered focal tracer uptake, likely attributed to traumatic etiology rather than metastatic, for clinical and radiological correlation.
- Mild degenerative changes are noticed at different levels of the spine and around big joints.

IMPRESSION:

- Worrisome tracer uptake pattern along both left femoral and tibial shafts, for clinical/radiological correlation.

♦ *Much oblige*
♦ *Dr. Alaa Nagi, MD*
♦ *Consultant of nuclear medicine*

تاريخ الفحص: 22/10/2025

كود المريض: 68371

Thank you for referring this

منى رضوان سعيد

44

د/مروى نبيل

الاسم:

السن:

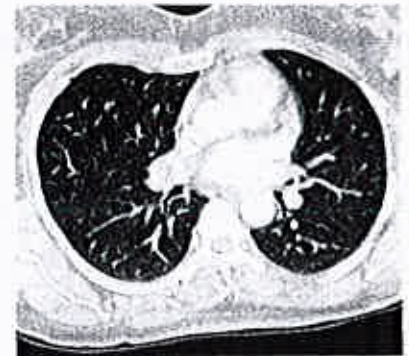
الطبيب المعالج أ.د.:

POST-CONTRAST MDCT OF THE CHEST, ABDOMEN & PELVIS REVEALED:

✦ A newly diagnosed case of right breast invasive duct carcinoma, for metastatic workup.

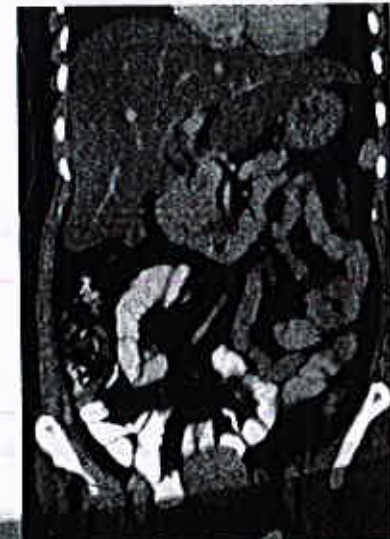
I-Chest:

- Two tiny subcentimetric pulmonary nodules are seen at the right middle lung lobe, not exceeding 5 mm in longest dimension.
- Tiny subpleural pulmonary nodule is also noted at the apicoposterior segment of left upper lung lobe, measuring 3 mm.
- No pulmonary ground glass or consolidative patches.
- No pulmonary masses or cavitary lesions.
- No evidence of pleural or pericardial sac collections.
- No evidence of mediastinal or hilar lymph node enlargement, masses or calcifications is seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Normal appearance of the thoracic aorta and the great vessels.
- **NB.** Right breast upper outer irregular shaped soft tissue lesion with associated enlarged ipsilateral axillary lymph nodes that shows cortical thickening; in keeping with the pathology proven invasive duct carcinoma.



II- Abdomen and pelvis:

- The liver is enlarged in size showing smooth regular outline and homogeneous hypodense parenchyma with no evidence of focal lesions or IHBRs dilatation. Geographic hyperdense area of fat sparing is seen adjacent to the gall bladder.
- Normal CT appearance of the spleen. No focal lesions.
- Normal appearance of the suprarenal glands, pancreas and both kidneys.
- Retro-aortic left renal vein is noted (anatomical variant).
- Normal CT appearance of pelvic organs.



BILATERAL BREAST ULTRASONOGRAPHY

Mrs./ MONA RADWAN SAEID

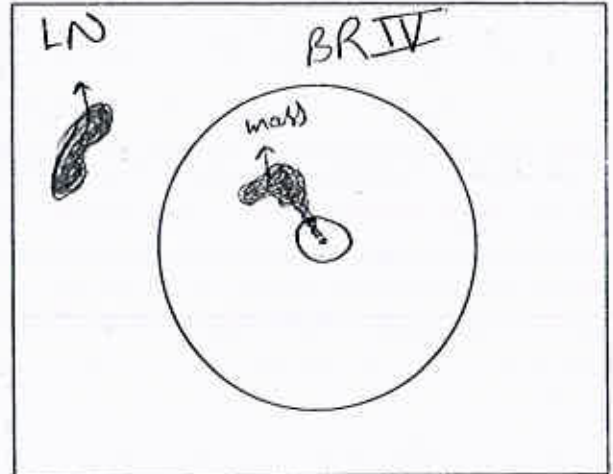
"10/9/2025"

Prof Dr: MARWA HANEM NABIL

9/10/2025

Right side:

- Slightly prominent fibro-glandular element of the breasts.
- Right upper outer quadrant well defined irregular hypoechoic mass lesion is seen measuring about (3.1 x 2.1 x 1.9 cm) in maximal diameters seen opposite 11-12 O'clock direction showing irregular outline. it casts mild posterior acoustic shadowing with hyperechoic halo seen around with central extension along the related lactiferous ducts.
- No evidence of sizeable cystic lesions.
- Average caliber of lactiferous ducts.
- Normal skin thickness.
- Normal appearance of nipple/areola complex.
- Pathological enlarged axillary LN with mild cortical thickening is noted measuring about (37 x 17 mm).



Left side:

- Normal fibro-glandular element of the breast.
- No evidence of sizeable solid or cystic lesions.
- Normal caliber of the lactiferous ducts.
- Normal skin thickness.
- Normal appearance of nipple/areola complex..
- No pathologically enlarged axillary lymph nodes.

IMPRESSION

- Right breast upper inner quadrant solid mass lesion as described BIRADS IV, recommended for histopathological assessment.
- No sizeable left breast solid or cystic mass lesions.

Tru-cut biopsy is available upon request.

Dr./Maged Barawy Dr./Ahmed Magdy MD.
Lecturer of radio-diagnosis

تاريخ الفحص :: 22/10/2025

كود المريض :: 68371

Thank you for referring this

منى رضوان سعيد

44

د/مروى نبيل

الاسم ::

السن ::

الطبيب المعالج أ.د.:

- No pelviabdominal ascites or lymphadenopathy.
- No evidence of any dorso-lumbar or pelvic osseous lesions.

Opinion:

A newly diagnosed case of right breast invasive duct carcinoma, for metastatic workup.

- Few subcentimetric non specific pulmonary nodules as described; for follow up.
- Fatty hepatomegaly, for laboratory correlation.
- No suspicious mediastinal or pelviabdominal lesions.

*Best Regards,
Dr. Joseph Gamil*

Patient ID: 92677	Patient Name English: Mona Radwan Saied Abdallatif
Age: ٤٤ سنة, ٩ شهر	2026/01/18 11:46:53 AM :Report Date

Women's Health Outreach Program

Bilateral Sono mammography

Clinical Summary: 44 years old female patient with right breast mastalgia.

Detailed Findings

Right breast

- Heterogenous fibroglandular densities. (ACR:C)
- UOQ focal hyperdense asymmetry more demonstrated by DBT as irregular spiculated hyperdense mass lesion with cluster of suspicious microcalcifications, occupying area measuring 4 x 7 cm ,1.7 cm from nipple.
- Complementary US revealed 10:00 irregular ill-defined heterogenous hypoechoic mass lesion with connecting prominent duct reaching nipple, measuring 3 x 2.5 cm .0.5 from the skin.
- No dilated retroareolar ducts.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogenous fibroglandular densities. (ACR:C)
- No masses, asymmetries, distortions or suspicious calcifications.
- Complementary US revealed heterogenous fibroglandular parenchyma.
- No solid or cystic mass lesions
- No dilated retroareolar ducts.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Conclusion:

- **Right breast:** UOQ suspicious mass lesion as described (BIRADS 4) for histopathological examination
- **Left breast:** normal study. (BIRADS 1)

Much Obliged,
Dr. Esraa Elsayed
 Dr. Norhan Osama